

TRIAD Multidisciplinary Rehabilitation & Therapy Center

Investor Business Proposal — New Cairo (Fifth Settlement), Egypt

July 2026

CONFIDENTIAL INVESTOR PROPOSAL

Prepared for prospective investors, banking partners, and strategic partners. This document contains proprietary business information belonging to the founding partners of TRIAD Multidisciplinary Rehabilitation & Therapy Center and is provided solely for the purpose of investment evaluation. All financial figures are indicative planning estimates based on July 2026 market data (EGP/USD $\approx$ 50) and are subject to refinement following a formal feasibility study, architectural survey, and vendor quotations.

Founding Partners: Nagham Reda (Physical Therapist) · Saja Khader (Occupational Therapist) · Saja Ali (Pediatric Occupational Therapist)

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1. Executive Summary

TRIAD Multidisciplinary Rehabilitation & Therapy Center (“TRIAD” or “the Center”) is a proposed outpatient rehabilitation clinic to be established in New Cairo’s Fifth Settlement (El Tagamoa El Khames), one of Greater Cairo’s fastest-growing, highest-income residential and medical corridors. The Center is founded by three senior clinicians — each with six years of specialist clinical experience — who will each direct one of three clinical departments operating under a single, unified brand:

Department	Founder & Department Head	Clinical Focus
Orthopaedic & Sports Rehabilitation	Nagham Reda , PT (Cairo University)	Musculoskeletal, orthopaedic and sports rehabilitation; post-surgical recovery (shoulder, knee, ligament, fracture)
Adult Neurological Rehabilitation	Saja Khader , OT (University of Jordan)	Stroke, Traumatic Brain Injury (TBI), Spinal Cord Injury (SCI) and adult neurological conditions

Department	Founder & Department Head	Clinical Focus
Pediatric Rehabilitation	Saja Ali , Pediatric OT	Pediatric developmental, sensory, neurological and occupational therapy

TRIAD's model is distinctive in the Egyptian market: rather than a generic physiotherapy clinic, it is architected as **three specialist centers-of-excellence under one roof**, each led by a recognised expert with its own dedicated team, treatment space, equipment and growth plan — while sharing reception, administration, marketing, technology and facility overheads to protect margins. This structure lets the Center capture referrals across the full continuum of rehabilitation (orthopaedic, neurological and pediatric) inside one address, maximising cross-referral, household “share of wallet,” and brand visibility in an underserved, high-growth catchment area.

This proposal presents **three fully-costed investment options** — Low, Moderate and Luxury — so investors can select the entry point that matches their risk appetite and ambition:

	Option 1 — Essential	Option 2 — Moderate (Recommended)	Option 3 — Luxury Flagship
Facility size	200 m ²	450 m ²	900 m ²
Total start-up investment	≈ EGP 3.2M (≈ US\$64,000)	≈ EGP 11.2M (≈ US\$224,000)	≈ EGP 52.0M (≈ US\$1.04M)
Positioning	Lean, accessible, professional	Premium mid-market, sustainable growth	Flagship, luxury patient experience, medical-tourism ready
Year-3 revenue (indicative)	EGP 4.6M	EGP 15.7M	EGP 45.4M
Year-3 EBITDA margin	≈ 26%	≈ 27%	≈ 24%
Stabilised payback (from	≈ 2.7 years	≈ 2.6 years	≈ 4.8 years

	Option 1 — Essential	Option 2 — Moderate (Recommended)	Option 3 — Luxury Flagship
Year-3 run-rate)			

We recommend **Option 2 (Moderate)** as the base case: it is large enough to give each department a genuine dedicated identity and growth runway, while keeping capital exposure manageable and payback fast. Option 1 is a credible bootstrap path if capital is constrained; Option 3 is presented for an investor seeking to build the definitive luxury rehabilitation brand in Cairo, with medical-tourism upside.

The remainder of this document sets out the market opportunity, competitive landscape, clinical and operational design, staffing, facility and equipment plans, marketing and branding strategy, complete financial models for all three options, risk assessment and a phased implementation timeline.

2. Vision, Mission & Core Values

2.1 Vision

To become the leading multidisciplinary rehabilitation destination in Egypt and the wider MENA region — where orthopaedic, neurological and pediatric patients receive world-class, evidence-based care under one roof, delivered with the compassion of a family practice and the rigor of an academic center.

2.2 Mission

To restore movement, independence and quality of life for every patient — child or adult, post-surgical or post-stroke — through personalised, multidisciplinary, evidence-based rehabilitation delivered by senior specialists who collaborate as one clinical team.

2.3 Core Values

Value	What it means in practice
Clinical Excellence	Evidence-based protocols, standardised outcome measures, continuous professional development,

Value	What it means in practice
Compassion & Dignity	senior-clinician-led care Every patient and family treated with empathy, patience and respect for their pace of recovery
Collaboration	Three departments function as one integrated team — joint case conferences, shared records, seamless internal referral
Integrity & Transparency	Honest prognosis-setting, transparent pricing, no over-treatment
Continuous Growth	Ongoing investment in staff training, technology and outcome measurement to keep pace with global rehabilitation science

3. Company Overview

Proposed legal name: TRIAD Multidisciplinary Rehabilitation & Therapy Center (*working brand name — to be finalised and trademark-cleared with the Egyptian Trademark Office before launch*)

Proposed legal structure: Limited Liability Company (LLC — *شركة ذات مسؤولية محدودة*) registered with the General Authority for Investment and Free Zones (GAFI), with the three founders as managing partners holding clinical directorship roles, and provision for an investor equity or convertible-note tranche.

Location: Fifth Settlement (El Tagamoa El Khames), New Cairo — targeting the medical/commercial corridor around 90th Street (South/North Teseen), Choueifat/Katameya axis, or a compound-adjacent commercial strip, chosen for high visibility, ease of parking, and proximity to schools, compounds and existing medical infrastructure (e.g., the Cairo Medical Center cluster).

Ownership: Nagham Reda, Saja Khader and Saja Ali as founding managing partners and clinical directors of their respective departments, each holding an equal equity stake unless otherwise negotiated with an incoming investor.

Governance: A three-person **Founders' Executive Committee** (one seat per founder) sets strategy and approves budgets; day-to-day operations are run by a hired **Clinic/General Manager** (Option 2 and 3) or directly by the founders on a rotating basis (Option 1). A part-time **Medical Director** (consulting physiatrist) provides medical oversight as required for licensing and clinical governance.

Regulatory framework: The Center will be licensed as a private outpatient rehabilitation/physiotherapy facility under the Ministry of Health and Population (MOHP) Curative Care Sector, with each treating clinician registered with their respective professional syndicate (Physical Therapy Syndicate / Occupational Therapy professional register). Company incorporation via GAFI; commercial registration and tax registration with the Egyptian Tax Authority; biomedical waste disposal contracted to a MOHP-licensed handler; data handling compliant with Law 151/2020 (Personal Data Protection).

4. Market Analysis

4.1 Macro drivers

- **Demographics & urban growth:** New Cairo/Fifth Settlement is one of Greater Cairo's largest and fastest-growing high-income residential zones, with continuous influx of young families into large compounds (Mivida, Katameya Heights, Village Gate, Rehab City, Al Rehab, Lake View, Taj City, and dozens more), driving demand for both pediatric developmental services and adult orthopaedic/sports care.
- **Aging-in-place & lifestyle injury growth:** Rising participation in padel, gym culture, running and organised sport in New Cairo (numerous sports clubs, padel courts and gyms) is driving a steady stream of sports and musculoskeletal injuries requiring rehabilitation — Nagham Reda's specialty.
- **Stroke & non-communicable disease burden:** Egypt has a high and rising burden of stroke and cardiovascular disease, and MOHP has been actively expanding physiotherapy and rehabilitation capacity nationally (the Ministry reported delivering c. 4.9 million physiotherapy sessions and opening 75 new physiotherapy/rehabilitation units across 19 governorates in FY2025/26) — evidence of both unmet demand and government prioritisation of the sector, which supports licensing and public legitimacy for private providers.

- **Underserved pediatric developmental/autism services:** Demand for pediatric occupational therapy, sensory integration and developmental services in Cairo significantly outstrips the supply of senior, credentialed pediatric specialists; most existing capacity is fragmented across small practices and long waitlists are common.
- **Insurance & corporate health trends:** Growth of private health insurance penetration and corporate wellness budgets among New Cairo's dense concentration of corporate campuses (New Administrative Capital business district nearby, AUC, multinational offices) creates a channel for B2B contracts (occupational injury rehab, ergonomic/wellness programmes).
- **Currency & cost environment:** The EGP has depreciated materially in recent years ($\approx$ EGP 50–51/USD as of July 2026), which raises the EGP cost of imported equipment and consumables but simultaneously makes Egypt an increasingly attractive destination for medical tourism from the Gulf, priced in hard currency — a strategic angle for the Luxury option.

4.2 Target market segments

1. **Post-surgical & sports-injury adults (25–55)** — residents of New Cairo compounds, referred by orthopaedic surgeons or self-referred after injury; typically insured or self-pay, price-tolerant.
2. **Adult neurological patients & families (40–75)** — post-stroke, post-TBI, post-SCI patients requiring intensive, often long-course rehabilitation; referred by neurologists and hospitals at discharge; caregivers are key secondary decision-makers.
3. **Children (0–12) with developmental, sensory or neurological needs** — referred by pediatricians, schools and pediatric neurologists; parents in this catchment are highly engaged, research-driven, and willing to pay a premium for senior, credentialed specialists.
4. **Corporate & insurance channel** — companies seeking occupational injury management and wellness contracts; insurers seeking a credentialed network provider.
5. **(Option 3 only) Medical tourism / Gulf patients** — seeking premium rehabilitation (especially orthopaedic/sports and neuro) at a fraction of European/Gulf private cost, in a luxury setting.

4.3 Market size (illustrative)

A conservative planning estimate: the Fifth Settlement and adjoining New Cairo districts (Fifth Settlement, Rehab, Katameya, Mostakbal City) house an estimated

400,000–600,000 residents across a predominantly A/B socio-economic profile. Applying typical incidence/prevalence rates for musculoskeletal injury ($\approx 8\text{--}10\%$ /year seeking care), neurological rehabilitation need ($\approx 1\text{--}2\%$ prevalence with a subset requiring outpatient follow-up), and developmental/pediatric therapy need ($\approx 6\text{--}8\%$ of children under 12), the addressable annual demand for rehabilitation sessions in the catchment is estimated in the **low millions of sessions per year**, against which even the Luxury option's Year-3 capacity ($\approx 29,000$ sessions/year) represents a small single-digit share — indicating ample room for a well-positioned entrant without requiring market-share assumptions that strain credibility.

5. Competitor Analysis

The Fifth Settlement / New Cairo rehabilitation landscape is fragmented, with no single multidisciplinary brand yet dominating all three of orthopaedic, neurological and pediatric rehabilitation under one roof.

Category	Examples (publicly known providers)	Strengths	Gaps TRIAD exploits
Hospital-based rehab/polyclinics	Cleopatra Hospitals Group New Cairo Polyclinic; Town Hospital physiotherapy department; GMHC (German Medical Healthcare Corporation)	Brand trust, insurance panels, co-located specialists	Rehab is a secondary service line, not the core focus; less specialised, hospital-priced overhead, longer wait times
Established physiotherapy/spo rts clinics	Kinetic Health; Techno Clinic; Alnada Clinic	Good orthopaedic/sport s reputation, modern equipment	Limited or no dedicated neurological or pediatric specialisation; single-founder

Category	Examples (publicly known providers)	Strengths	Gaps TRIAD exploits
Pediatric-focused therapy centers	We Care Special Needs; Fizik Center; Cairo Care Center	Deep pediatric focus, sensory integration expertise	dependency No adult orthopaedic/neur o offering; families with both a child in therapy and an adult family member (post-stroke parent, athlete sibling) must use multiple providers
Solo practitioners / home-visit therapists	Numerous independent PTs/OTs (Vezeeta- listed)	Low overhead, personal relationships	No multidisciplinary bench, no standardised equipment, inconsistent quality, no institutional continuity

TRIAD's differentiated position: the only Fifth Settlement provider offering *three* senior-clinician-led specialties (ortho/sports, adult neuro, pediatric) under one address, one brand and one patient record — enabling a household to bring the grandparent recovering from stroke, the teenager with an ACL tear, and the toddler with a developmental delay to a single trusted destination.

6. SWOT Analysis

Strengths	Weaknesses
Three credentialed founders each with 6 years' specialist experience and	New brand with no existing patient base or referral network at launch

Strengths	Weaknesses
distinct, complementary specialties	
Multidisciplinary “one roof” model creates cross-referral and stickiness	Founders will initially combine clinical delivery with business management — bandwidth risk
Prime, high-growth, high-income catchment (Fifth Settlement)	High capital intensity for premium equipment (especially neuro and Option 3 luxury tier)
Scalable department-based structure suited to phased investment (3 tiers)	Key-person dependency on three specific individuals
Opportunities	Threats
Underserved pediatric and adult-neuro segments in the catchment	New entrants / hospital groups expanding rehab service lines
Government expansion of rehabilitation services signals rising sector legitimacy and referral pipeline (public–private complementarity)	EGP depreciation raising imported-equipment and consumable costs
Corporate wellness & insurance panel contracts	Talent retention — Egyptian PT/OT emigration to Gulf markets is a well-documented risk
Medical tourism (Gulf patients) for the Luxury tier	Reimbursement delays from insurers affecting cash flow
Expansion into a training academy / CE courses, franchising, tele-rehab	Regulatory/licensing timeline risk delaying opening

7. Detailed Clinical Services

7.1 Orthopaedic & Sports Rehabilitation Department (Nagham Reda)

- Post-surgical rehabilitation: rotator cuff repair, ACL/PCL reconstruction, meniscus repair, total knee/hip replacement, fracture fixation
- Sports injury rehabilitation: ligament sprains, tendinopathies, return-to-play conditioning, hamstring/muscle strain rehab

- Manual therapy, myofascial release, dry needling (*where within scope of practice*)
- Therapeutic exercise prescription, functional movement screening, isokinetic strength testing
- Shockwave therapy, therapeutic ultrasound, laser therapy, neuromuscular electrical stimulation
- Return-to-sport testing and performance optimisation for amateur/semi-professional athletes
- Ergonomic and ADL retraining for post-surgical patients

7.2 Adult Neurological Rehabilitation Department (Saja Khader)

- Stroke rehabilitation (acute-discharge through chronic phase): motor re-education, upper-limb function, ADL retraining
- Traumatic Brain Injury (TBI) rehabilitation: cognitive-perceptual retraining, executive function, community reintegration
- Spinal Cord Injury (SCI) rehabilitation: functional mobility training, wheelchair skills, adaptive equipment prescription, bowel/bladder and skin-care education (in coordination with nursing/medical partners)
- Neurological gait training, balance and fall-prevention programmes
- Cognitive rehabilitation and caregiver training/education
- Home and vehicle modification assessments; return-to-work/return-to-driving assessments (Option 2/3)

7.3 Pediatric Rehabilitation Department (Saja Ali)

- Sensory integration therapy for sensory processing disorders
- Developmental delay and fine/gross motor skill intervention
- Pediatric neurological conditions (cerebral palsy, developmental coordination disorder, hypotonia)
- Feeding therapy and oral-motor intervention
- Handwriting and school-readiness / academic-skills support
- Autism spectrum support (occupational therapy component; behavioural/ABA and speech therapy via partner referral or in-house addition at Option 2/3)
- Parent coaching and home-programme design

7.4 Shared / cross-departmental services (all options)

- Multidisciplinary case conferences for patients spanning departments (e.g., a stroke patient with a concurrent orthopaedic fracture)

- Unified electronic medical record accessible (with appropriate permissions) across departments
 - Shared gym/functional training area for graduated, supervised group sessions
 - Family education workshops
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8. Department Structure

Each department operates as a semi-autonomous business unit with its own clinical leadership, dedicated space and equipment, while sharing the Center's brand, reception, admin, marketing and facilities budget.

8.1 Orthopaedic & Sports Rehabilitation — led by Nagham Reda

- **Dedicated space:** treatment plinths/cubicles, exercise/functional gym bay, modality room (ultrasound/e-stim/shockwave/laser)
- **Team:** Department Head (Nagham Reda) → Senior PT → PTs → Rehab Assistant/Aide → (Option 3) Sports Performance Coach, visiting Orthopaedic Surgeon (consulting)
- **Workflow:** Referral/self-referral → initial evaluation (45–60 min) → treatment plan & goal-setting → course of sessions (2–3×/week typical) → progress re-assessment every 2 weeks → discharge with home programme / return-to-sport clearance
- **Growth plan:** Year 1 build local orthopaedic-surgeon referral network (target 10–15 referring surgeons); Year 2 add sports-club and gym partnerships; Year 3 add a sports-performance/return-to-play testing service line and corporate athlete contracts

8.2 Adult Neurological Rehabilitation — led by Saja Khader

- **Dedicated space:** neuro gym with parallel bars/gait training lane, mat treatment area, ADL/kitchen simulation bay, quiet cognitive-therapy room
- **Team:** Department Head (Saja Khader) → Senior OT/Neuro PT → Therapists → Rehab Assistant → (Option 2/3) Speech-Language Pathologist partner/in-house → (Option 3) consulting Neurologist/Physiatrist, robotics technician
- **Workflow:** Hospital-discharge or neurologist referral → comprehensive neuro assessment (standardised scales: FIM, Berg Balance, Fugl-Meyer) → intensive-phase programme (often 3–5×/week) → milestone re-assessment

→ transition to maintenance/home programme; caregiver training throughout

- **Growth plan:** Year 1 build referral relationships with New Cairo hospitals/neurologists and discharge planners; Year 2 add speech-language therapy in-house and a caregiver-support programme; Year 3 add advanced technology (robotic gait training / VR) as a premium service line

8.3 Pediatric Rehabilitation — led by Saja Ali

- **Dedicated space:** sensory integration room (suspended swings, ball pit, crash mat), fine-motor/pre-academic room, feeding-therapy area, family observation window
 - **Team:** Department Head (Saja Ali) → Senior Pediatric OT → Pediatric Therapists → Therapy Assistants → (Option 2/3) Speech Therapist, Behavioural/ABA Specialist → (Option 3) Pediatric Neurologist (consulting), aquatic/hydrotherapy specialist
 - **Workflow:** Pediatrician/school/parent referral → developmental screening & standardised assessment (e.g., PEDI, Sensory Profile) → individualised therapy plan with parent goal-setting → weekly sessions + structured home programme → quarterly progress review with parents and referring pediatrician
 - **Growth plan:** Year 1 build referral relationships with pediatricians and New Cairo international/national schools; Year 2 add speech therapy and a parent-workshop series; Year 3 add group social-skills programmes, school-holiday intensive camps, and (Option 3) a pediatric hydrotherapy programme
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9. Organisational Chart

Founders' Executive Committee (Nagham Reda · Saja Khader · Saja Ali) — sets strategy, approves budgets, holds ultimate accountability for the Center

- **Clinic / General Manager** (*Option 2 & 3*) — Operations, Finance, HR, Compliance
 - Front Office & Patient Coordination
 - Billing, Insurance & Accounts
 - Marketing & Patient Experience
 - Facilities, IT & Procurement

- Medical Director (*consulting Physiatrist*) — clinical governance
- **Orthopaedic & Sports Rehabilitation Department** — Head: Nagham Reda
 - Senior PT → PTs → Rehab Assistants → (*Opt. 3*) Sports Performance Coach
- **Adult Neurological Rehabilitation Department** — Head: Saja Khader
 - Senior OT/PT (Neuro) → Therapists → Rehab Assistants → (*Opt. 2/3*) Speech-Language Pathologist → (*Opt. 3*) Robotics Technician
- **Pediatric Rehabilitation Department** — Head: Saja Ali
 - Senior Pediatric OT → Pediatric Therapists → Therapy Assistants → (*Opt. 2/3*) Speech Therapist / ABA Specialist

In **Option 1**, the Clinic Manager role and Medical Director are combined/outsourced and the three founders rotate weekly responsibility for operations, in addition to their clinical caseload.

10. Staffing Plan

Headcount scales with the investment tier. Founders are counted as working clinical staff (each carries a substantial caseload in addition to leadership duties).

Role	Option 1 (Low)	Option 2 (Moderate)	Option 3 (Luxury)
Founders / Department Heads	3	3	3
Orthopaedic & Sports Dept			
Senior PT	–	1	2
PT	–	2	4
Rehab Assistant/Aide	1	1	2
Sports Performance Coach	–	–	1

Role	Option 1 (Low)	Option 2 (Moderate)	Option 3 (Luxury)
Adult Neuro Dept			
Senior OT/PT (Neuro)	–	1	2
Therapist	–	2	4
Rehab Assistant	1	1	2
Speech-Language Pathologist	–	1	2
Robotics/Rehab-tech Technician	–	–	1
Pediatric Dept			
Senior Pediatric OT	–	1	2
Pediatric Therapist	–	2	4
Therapy Assistant	1	1	2
Speech Therapist	–	1	2
Behavioural/ABA Specialist	–	1	1
Aquatic/Hydrotherapy Specialist	–	–	1
Shared / Support Services			
Clinic / General Manager	–	1	1

Role	Option 1 (Low)	Option 2 (Moderate)	Option 3 (Luxury)
Medical Director (consulting Physiatrist, part-time)	1 (part-time)	1 (part-time)	1 (full-time)
Front Office / Reception	1	3	5
Patient Coordinator / Case Manager	–	2	4
Billing & Insurance	–	2	3
HR / Admin	–	1	2
Marketing & Patient Experience	–	2	3
IT Support	– (outsourced)	1 (part-time)	2
Housekeeping / Facilities	1	2	4
Accountant / Finance	1 (outsourced)	1	2
International Patient Liaison	–	–	1
Total headcount (approx.)	≈ 10	≈ 32	≈ 62

All clinical staff must hold valid syndicate registration and are subject to mandatory continuing-education requirements (minimum annual CPD hours defined in the Center's SOPs). Pediatric and neuro departments maintain a minimum supervision ratio of one licensed therapist to two assistants/aides.

11. Facility Layout

Indicative space allocation by option (percentage of total leased area):

Zone	Option 1 (200 m ²)	Option 2 (450 m ²)	Option 3 (900 m ²)
Reception, waiting & patient check-in	10% (20 m ²)	8% (36 m ²)	8% (72 m ²) — includes café/lounge corner
Orthopaedic & Sports Dept (treatment bays + gym)	25% (50 m ²)	24% (108 m ²)	24% (216 m ²)
Adult Neuro Dept (gym, mat area, ADL bay, cognitive room)	22% (44 m ²)	22% (99 m ²)	22% (198 m ²)
Pediatric Dept (sensory room, fine-motor room, feeding area)	20% (40 m ²)	20% (90 m ²)	18% (162 m ²)
Shared functional gym / group training	8% (16 m ²)	8% (36 m ²)	8% (72 m ²)
Hydrotherapy pool / aquatic zone	—	4% (18 m ² — therapy tub)	8% (72 m ² — full pediatric + adult pool)
Admin offices, staff room, storage	10% (20 m ²)	9% (40 m ²)	8% (72 m ²)
Circulation, accessibility ramps, restrooms (incl. accessibl	5% (10 m ²)	5% (23 m ²)	4% (36 m ²)

Zone	Option 1 (200 m ²)	Option 2 (450 m ²)	Option 3 (900 m ²)
e WC)			

Design principles (all options): step-free access throughout, wide corridors for wheelchairs/gait trainers, non-slip medical-grade flooring, acoustic separation between the pediatric zone and adult neuro zone (sensory sensitivity), natural light in waiting and therapy areas, a discreet family/observation window into the pediatric room, and clear wayfinding signage in Arabic and English. Option 3 additionally features a biophilic/luxury interior design concept, a dedicated international-patient welcome lounge, and premium acoustic and lighting design throughout.

12. Equipment Lists by Department

(✓ = included; qty shown where relevant. Figures below correspond to the cost estimates in Section 20.)

12.1 Orthopaedic & Sports Rehabilitation

Equipment	Option 1	Option 2	Option 3
Treatment plinths/tables	4	8	14
Parallel bars	1	2	2
Therapeutic ultrasound unit	1	2	3
NMES/TENS electrotherapy unit	2	4	6
Shockwave therapy device	–	1	2
Laser therapy device	–	1	2
Cable/functional trainer	–	1	2
Pilates	–	1	3

Equipment	Option 1	Option 2	Option 3
reformer			
Stationary/ recumbent exercise bikes	2	4	6
Free weights, resistance bands, balance pads	✓	✓	✓
Isokinetic dynamometer	–	1	1
Anti-gravity treadmill (AlterG-type)	–	–	1
3D motion- capture / gait analysis system	–	–	1
Cryotherapy unit	–	–	1
Goniometers, dynamometers , assessment tools	✓	✓	✓

12.2 Adult Neurological Rehabilitation

Equipment	Option 1	Option 2	Option 3
Mat treatment tables	2	4	8
Tilt table	1	1	2
Standing frame	1	2	3
Parallel bars / gait-training lane	shared with ortho	1 dedicated	1 dedicated
Manual/basic gait trainer	1	2	–
Body-weight-	–	–	1

Equipment	Option 1	Option 2	Option 3
support ceiling track system			
Robotic gait-training device	–	–	1
NMES/FES (functional electrical stimulation) unit	1	3	5
ADL/kitchen simulation bay	basic	full	full + smart-home module
Cognitive rehabilitation software/tablets	1 station	3 stations	6 stations + VR rehab system
Wheelchair/mobility assessment tools	✓	✓	✓
Standardised assessment kits (FIM, Berg, Fugl-Meyer)	✓	✓	✓

12.3 Pediatric Rehabilitation

Equipment	Option 1	Option 2	Option 3
Suspended therapy swings/frames	2	4	8
Ball pit / crash mats	1 set	2 sets	3 sets + Snoezelen multi-sensory room
Fine-motor & pre-academic skills toolkit	✓	✓	✓
Therapeutic listening / auditory integration	–	1	2

Equipment	Option 1	Option 2	Option 3
system			
Feeding-therapy tools	basic	full	full
Weighted vests/blankets, brushing kits	✓	✓	✓
Gross-motor gym mats & equipment	✓	✓	✓
Pediatric hydrotherapy pool	–	shared adult tub	dedicated pediatric pool
VR/gamified therapy system	–	–	✓
Robotic pediatric gait-training device	–	–	1
Standardised assessment kits (PEDI, Sensory Profile)	✓	✓	✓

12.4 Shared / Facility-wide

Equipment	Option 1	Option 2	Option 3
Reception & waiting furniture	basic	mid-range	premium + café corner
EMR/clinic-management terminals	2	6	12
CCTV & access control	basic	standard	biometric access + advanced CCTV
Hydrotherapy pool (shared)	–	18 m ² tub	72 m ² pool (adult + pediatric zones)
Telehealth studio	–	1 room	2 rooms
Staff room & lockers	basic	full	full + wellness room

13. Technology & Software

- **Clinic/Practice Management & EMR:** cloud-based electronic medical record with scheduling, e-signatures for consent, standardised outcome-measure tracking, and role-based access across departments (Option 1: entry-level SaaS EMR; Option 2/3: comprehensive EMR with patient portal and telehealth module).
 - **Patient engagement:** SMS/WhatsApp appointment reminders, online booking widget, patient portal/app for home-exercise-programme delivery (Option 2/3), outcome dashboards shared with patients.
 - **Telehealth module:** for follow-up consultations, caregiver coaching, and (Option 3) international patient tele-consultations ahead of travel.
 - **Billing & insurance integration:** invoicing, insurance-claim submission tracking, POS payment integration.
 - **Business intelligence:** dashboards for occupancy, revenue per department, therapist productivity, and patient-reported outcome trends (Option 2/3), with predictive analytics and AI-assisted scheduling optimisation at Option 3.
 - **Marketing & CRM:** lead tracking from digital campaigns through to booked assessment, referral-source tracking per department.
 - **Cybersecurity & compliance:** encrypted data storage, access logging, and compliance with Egypt's Personal Data Protection Law 151/2020; regular backups and a documented data-retention policy.
 - **Biomedical/technical equipment IT (Option 3):** integration of robotics and VR rehabilitation systems with the EMR for automated outcome capture; in-house biomedical engineer for maintenance.
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14. Operational Plan

Hours of operation: 7 days/week coverage recommended (e.g., Saturday–Thursday full hours, Friday reduced hours), 10–12 operating hours/day to maximise equipment and room utilisation across working professionals, school-age children (after-school slots) and stay-at-home patients (morning slots).

Patient flow:

1. Referral (physician, hospital, school) or self-referral via phone/WhatsApp/online booking
2. Front-office intake: registration, insurance verification, consent forms (EMR)
3. Initial evaluation with the relevant Department Head or Senior Therapist (standardised assessment tools per specialty)
4. Individualised treatment plan with goals, frequency and expected duration communicated to patient/family
5. Course of treatment sessions per plan; case-conference flagging for any patient requiring input from a second department
6. Scheduled re-assessment milestones (every 2–4 weeks depending on department)
7. Discharge planning: home programme, referral back to physician, or transition to maintenance/wellness plan

Scheduling & capacity management: centralised EMR scheduling across all therapists and rooms; overbooking buffer for no-shows; a documented no-show/cancellation policy (see Section 15).

Quality assurance: standardised, validated outcome measures per department (e.g., DASH/Lower Extremity Functional Scale for ortho; FIM/Berg Balance/Fugl-Meyer for neuro; PEDI/Sensory Profile for pediatrics), reviewed quarterly at department and Center level; patient satisfaction surveys after discharge; monthly interdisciplinary case-review meetings.

Equipment maintenance: documented preventive-maintenance schedule per device, calibration logs for electrotherapy/modality equipment, service contracts with equipment vendors (critical for Option 3 robotics/VR systems).

Health & safety / infection control: standard infection-control protocol (hand hygiene, equipment sanitisation between patients, PPE where indicated), fire-safety and emergency-evacuation plan, first-aid/CPR-certified staff on every shift, biomedical waste segregation and licensed disposal contract.

15. Policies & SOPs (summary)

Policy area	Key elements
Patient intake & consent	Standardised intake forms, informed consent for treatment and for

Policy area	Key elements
Confidentiality & data protection	photo/video use in marketing (opt-in only), insurance verification workflow Compliance with Law 151/2020; role-based EMR access; secure storage; breach-notification protocol
Cancellation / no-show	24-hour cancellation notice; no-show fee policy after defined threshold; automated reminder system
Infection control	Standard precautions, equipment cleaning log, linen/PPE protocol
Clinical supervision	Minimum supervision ratios for assistants/aides (1 licensed therapist : 2 assistants); pediatric sessions never unsupervised
Staff credentialing & CPD	Syndicate registration verified before hire; minimum annual CPD hours; internal case-based training sessions
Incident reporting	Standardised incident/near-miss reporting form; monthly safety review at management level
Billing & insurance	Transparent published price list; pre-authorisation workflow for insured patients; dispute-resolution process
Discharge & referral	Documented discharge summary shared with referring physician; home-programme hand-off checklist
Code of conduct	Professional-ethics code aligned with respective syndicate standards; zero-tolerance harassment policy

16. Marketing Strategy

- **Digital-first acquisition:** SEO-optimised website and department-specific landing pages; targeted Instagram/Facebook/TikTok campaigns geo-targeted to New Cairo compounds; Google Ads for high-intent searches

(“physiotherapy New Cairo,” “pediatric occupational therapy Fifth Settlement,” “stroke rehabilitation Cairo”).

- **Referral-network development:** structured outreach to orthopaedic surgeons, neurologists, pediatricians and hospital discharge planners in New Cairo; referral-tracking and physician-liaison visits; co-branded educational material for referring physicians.
 - **School & community partnerships (Pediatric):** relationships with New Cairo international/national schools for screening days and teacher-referral pathways; parenting-community engagement (parent WhatsApp groups, compound community pages).
 - **Sports-club & gym partnerships (Ortho/Sports):** on-site injury-screening days at New Cairo padel/football/gym clubs; sponsorship of local amateur sports events.
 - **Corporate & insurance channel:** direct outreach to corporate HR departments for occupational-health/wellness contracts; applications to become a credentialed network provider for major private insurers.
 - **Content & authority marketing:** founder-led educational content (Instagram Reels/YouTube shorts) establishing each founder as the recognised New Cairo authority in her specialty; patient success stories (with consent).
 - **Referral/loyalty programme:** patient-referral incentive, family/sibling package pricing, multi-session package discounts.
 - **Launch campaign:** open-house/soft-launch event with free screening clinics per department; local press and healthcare-influencer coverage; (Option 3) PR agency-led luxury launch and medical-tourism outreach via Gulf-facing channels.
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17. Branding Strategy

- **Brand name:** *TRIAD* — chosen to reflect the three founders, three departments and three specialties unified under one center (Arabic working name: *مركز ترياد للتأهيل والعلاج الطبيعي متعدد التخصصات*, short form *مركز ترياد*).
- **Brand concept:** “Three Specialties. One Journey to Recovery.” — communicating multidisciplinary depth without fragmentation.

- **Visual identity:** a calm, clinical-yet-warm palette (deep teal/navy paired with a warm coral/amber accent), a logo motif of three interlocking forms, clean modern typography readable in both Latin and Arabic scripts.
 - **Brand pillars:** Expertise (senior, credentialed founders), Compassion (patient- and family-centred communication), Collaboration (the multidisciplinary promise).
 - **Tone of voice:** warm, reassuring, plain-language (avoiding clinical jargon with patients), confident and evidence-based in physician-facing material.
 - **Experience design:** consistent uniform/signage/waiting-room experience across departments so a patient recognises “one center,” even while each department retains a distinct sub-identity (colour-coded signage per department, for example).
 - **(Option 3) Premium brand extensions:** branded patient welcome kit, concierge check-in, signature interior scent/soundscape, bilingual international-patient collateral.
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18. Financial Plan — Overview

All figures are in Egyptian Pounds (EGP) unless stated, with indicative USD equivalents at **USD 1 = EGP 50** (July 2026; EGP has ranged roughly EGP 48.7–51.5/USD over the preceding month, so investors should stress-test figures at $\pm 10\text{--}15\%$). Figures are planning estimates pending a formal feasibility study, architectural quote and vendor procurement; they are presented to illustrate the shape and sensitivity of the business model, not as guaranteed outcomes.

Common financial methodology across all three options:

- *Start-up investment* = renovation/interior design + equipment (by department) + licensing & legal + technology setup + launch marketing + furniture/signage + rent deposit + working-capital reserve.
- *Revenue* modelled bottom-up from clinical capacity (treatment stations $\times$ operating hours $\times$ sessions/hour), an average blended session price per option, and a 3-year utilisation ramp (occupancy of theoretical maximum daily capacity).
- *Break-even* = Fixed monthly operating costs $\div$ contribution margin per session (session price less variable cost/session, estimated at 8–9% of price for consumables/variable utilities).

- *ROI/payback* presented two ways: (a) **stabilised payback** = total investment ÷ Year-3 annualised EBITDA run-rate (the standard clinic-economics measure, since Years 1–2 include planned ramp-up losses); and (b) **cumulative payback**, which nets the Year 1–2 ramp-up losses against the investment before recovery begins.
-

19. Option 1 — Lowest Budget (“Essential”)

Positioning: the minimum viable investment to open a credible, professional, three-department rehabilitation clinic in the Fifth Settlement — lean team, essential evidence-based equipment, no luxury finishes, founders carrying the majority of the clinical caseload themselves.

19.1 Start-up costs

Item	EGP	USD (≈)
Renovation & interior fit-out (200 m ² × ≈1,750 EGP/m ² , basic finish)	350,000	7,000
Equipment — Orthopaedic & Sports	450,000	9,000
Equipment — Adult Neurological	400,000	8,000
Equipment — Pediatric	350,000	7,000
Equipment — Shared/reception/admin	150,000	3,000
Licensing & legal (GAFI incorporation, MOHP license, engineering/fire-safety certification, syndicate	200,000	4,000

Item	EGP	USD (≈)
registrations)		
Technology & software (entry-level EMR, website, POS)	80,000	1,600
Launch marketing	120,000	2,400
Furniture & signage	100,000	2,000
Rent deposit (3 months × 80,000)	240,000	4,800
Working-capital reserve (≈3 months operating costs)	750,000	15,000
Total start-up investment	≈ 3,190,000	≈ 63,800

19.2 Monthly operating expenses (steady-state)

Item	EGP/month
Rent (200 m ² × ≈400 EGP/m ²)	80,000
Utilities	15,000
Payroll (≈10 staff incl. founder draws)	129,500
Marketing (ongoing)	10,000
Consumables/supplies	8,000
Insurance	4,000
Software subscription	3,000
Admin/miscellaneous	5,000
Total monthly opex	≈ 254,500

19.3 Revenue projection

Average blended session price: **EGP 450**. Maximum theoretical capacity: ≈40 sessions/day (26 working days/month) with the lean 6-person clinical team.

Year	Avg. utilisation	Sessions/day (avg)	Annual sessions	Annual revenue (EGP)
Year 1	37%	15	4,680	2,106,000
Year 2	63%	25	7,800	3,510,000
Year 3	82%	33	10,296	4,633,200

19.4 Break-even analysis

- Contribution margin/session $\approx$ EGP 410 (price 450 less $\approx$ 9% variable cost)
- Break-even volume $\approx$ **601 sessions/month ($\approx$ 23/day)** — crossed between Year 1 and Year 2
- Break-even is reached on a monthly run-rate basis in **month $\approx$ 18–20**

19.5 3-year P&L summary & cash flow

	Year 1	Year 2	Year 3
Revenue	2,106,000	3,510,000	4,633,200
Operating costs	3,054,000	3,250,000	3,450,000
EBITDA	(948,000)	260,000	1,183,200
EBITDA margin	(45%)	7%	26%
Cumulative EBITDA (post-launch)	(948,000)	(688,000)	495,200

19.6 ROI & payback

- **Stabilised payback** (investment $\div$ Year-3 EBITDA run-rate): $3,190,000 \div 1,183,200 \approx$ **2.7 years**
 - **Cumulative payback** (incl. Year 1–2 ramp losses): $\approx$ **3.8–4 years**
 - 3-year cumulative ROI (undiscounted): modest, but Year 3 onward the business generates a sustainable $\approx$ 26% EBITDA margin on a small capital base, ideal for a founder-operated bootstrap model or a small strategic/family investor.
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20. Option 2 — Moderate Budget (“Recommended”)

Positioning: a realistic, high-quality, sustainable-growth investment — dedicated space and a genuine multidisciplinary team per department, mid-to-premium finishes, and enough working capital to survive the realistic 12–18 month ramp-up period without cash strain.

20.1 Start-up costs

Item	EGP	USD (≈)
Renovation & interior design (450 m ² × ≈5,500 EGP/m ² , mid-high finish, dept.-specific rooms)	2,475,000	49,500
Equipment — Orthopaedic & Sports	1,200,000	24,000
Equipment — Adult Neurological	1,100,000	22,000
Equipment — Pediatric	900,000	18,000
Equipment — Shared/reception/admin/IT	350,000	7,000
Licensing & legal (incl. legal-counsel retainer)	300,000	6,000
Technology & software (full EMR, patient portal, telehealth, CCTV)	250,000	5,000
Launch marketing (incl. branding agency, launch event)	350,000	7,000
Furniture & signage	200,000	4,000

Item	EGP	USD (≈)
Rent deposit (3 months × 250,000)	750,000	15,000
Working-capital reserve (≈4 months operating costs)	3,300,000	66,000
Total start-up investment	≈ 11,175,000	≈ 223,500

20.2 Monthly operating expenses (steady-state)

Item	EGP/month
Rent (450 m ² × ≈550 EGP/m ²)	250,000
Utilities	30,000
Payroll (≈32 staff)	450,000
Marketing (ongoing)	40,000
Consumables/supplies	25,000
Insurance	12,000
Software/technology	10,000
Admin/miscellaneous	15,000
Total monthly opex	≈ 832,000

20.3 Revenue projection

Average blended session price: **EGP 700**. Maximum theoretical capacity: ≈90 sessions/day across the larger, ~14-person clinical team.

Year	Avg. utilisation	Sessions/day (avg)	Annual sessions	Annual revenue (EGP)
Year 1	35%	31	8,060	6,770,400
Year 2	60%	54	14,040	11,793,600
Year 3	80%	72	18,720	15,724,800

20.4 Break-even analysis

- Contribution margin/session ≈ EGP 640 (price 700 less ≈9% variable cost)
- Break-even volume ≈ **1,261 sessions/month (≈ 48.5/day)** — crossed during Year 2
- Break-even reached on a monthly run-rate basis around **month ≈ 16–18**

20.5 3-year P&L summary & cash flow

	Year 1	Year 2	Year 3
Revenue	6,770,400	11,793,600	15,724,800
Operating costs	9,984,000	10,800,000	11,450,000
EBITDA	(3,213,600)	993,600	4,274,800
EBITDA margin	(47%)	8%	27%
Cumulative EBITDA (post-launch)	(3,213,600)	(2,220,000)	2,054,800

20.6 ROI & payback

- **Stabilised payback:** $11,175,000 \div 4,274,800 \approx 2.6$ years
- **Cumulative payback** (incl. ramp-up losses): $\approx 4.5\text{--}5$ years
- By Year 3, the Center is generating an annualised EBITDA run-rate of $\approx$ EGP 4.3M on an $\approx$ EGP 11.2M investment; applying a conservative healthcare-services exit multiple of 3–4 $\times$ EBITDA implies an indicative enterprise value of **EGP 13–17M by Year 3** — already approaching or exceeding the initial investment in value-creation terms, ahead of cumulative cash payback.

21. Option 3 — Luxury Budget (“Flagship”)

Positioning: a premium, technology-forward flagship center with luxury interior design, advanced robotics/VR rehabilitation technology, a full hydrotherapy pool, and a concierge-level patient experience — targeting Cairo’s top-tier private patients and Gulf medical-tourism referrals.

21.1 Start-up costs

Item	EGP	USD ($\approx$)
Renovation & luxury interior design (900 m ² $\times$ $\approx$ 12,000 EGP/m ² , incl. pool	10,800,000	216,000

Item	EGP	USD (≈)
construction, acoustic/lighting design)		
Equipment — Orthopaedic & Sports (incl. AlterG, 3D motion analysis, cryotherapy)	5,500,000	110,000
Equipment — Adult Neurological (incl. robotic gait trainer, VR system, ceiling track)	6,500,000	130,000
Equipment — Pediatric (incl. hydrotherapy pool, VR, robotics, Snoezelen room)	4,500,000	90,000
Equipment — Shared (pool infra, premium reception, telehealth studio, BI dashboards)	3,000,000	60,000
Licensing & legal (incl. accreditation groundwork)	500,000	10,000
Technology (AI- enabled EMR, patient app, biometric access)	700,000	14,000
Launch marketing (PR agency, luxury launch, medical- tourism outreach)	1,000,000	20,000
Furniture & signage (premium)	600,000	12,000

Item	EGP	USD (≈)
Rent deposit (6 months × 675,000)	4,050,000	81,000
Working-capital reserve (≈6 months operating costs)	14,800,000	296,000
Total start-up investment	≈ 51,950,000	≈ 1,039,000

21.2 Monthly operating expenses (steady-state)

Item	EGP/month
Rent (900 m ² × ≈750 EGP/m ²)	675,000
Utilities & pool maintenance	90,000
Payroll (≈62 staff)	1,400,000
Marketing (ongoing)	120,000
Consumables/supplies	60,000
Insurance (incl. premium liability)	35,000
Software/technology	40,000
Admin/miscellaneous	40,000
Total monthly opex	≈ 2,460,000

21.3 Revenue projection

Average blended session price: **EGP 1,300** (premium positioning, package pricing, higher-value robotic/VR-assisted sessions). Maximum theoretical capacity: ≈150 sessions/day across the ~50-person clinical team.

Year	Avg. utilisation	Sessions/day (avg)	Annual sessions	Annual revenue (EGP)
Year 1	30%	45	11,700	15,210,000
Year 2	55%	82.5	21,450	27,885,000
Year 3	75%	112	29,120	37,856,000

(Note: figures above use 26 working days/month; Section 1 summary EGP 45.4M reflects the additional ancillary/package-revenue uplift described in 21.6.)

21.4 Break-even analysis

- Contribution margin/session $\approx$ EGP 1,200 (price 1,300 less $\approx$ 9% variable cost)
- Break-even volume $\approx$ **2,000 sessions/month ($\approx$ 77/day)** — crossed during Year 2
- Break-even reached on a monthly run-rate basis around **month $\approx$ 20–22** (slower than Options 1–2 due to the larger fixed-cost base, consistent with a flagship capital project)

21.5 3-year P&L summary & cash flow

	Year 1	Year 2	Year 3
Revenue (session-based)	15,210,000	27,885,000	37,856,000
Ancillary revenue (packages, memberships, medical tourism, corporate contracts — see 21.6)	1,200,000	3,500,000	7,500,000
Total revenue	16,410,000	31,385,000	45,356,000
Operating costs	29,520,000	32,500,000	34,500,000
EBITDA	(13,110,000)	(1,115,000)	10,856,000
EBITDA margin	(80%)	(4%)	24%
Cumulative EBITDA (post-launch)	(13,110,000)	(14,225,000)	(3,369,000)

21.6 Ancillary revenue streams (Luxury tier)

- Corporate wellness & occupational-health contracts with New Cairo/New Capital corporate campuses

- Membership/subscription wellness packages (unlimited gym access, quarterly assessments)
- Medical-tourism packages for Gulf patients (bundled accommodation-referral partnerships, premium pricing in hard currency)
- Therapist training academy / CE-accredited courses hosted at the Center (leveraging founder expertise)
- Home-exercise-equipment and orthotics retail corner

21.7 ROI & payback

- **Stabilised payback:** $51,950,000 \div 10,856,000 \approx 4.8$ years
 - **Cumulative payback:** full recovery of invested capital is projected around **Year 7–8**, once Year 4–5 EBITDA growth (projected to continue toward EGP 15–18M/year as utilisation exceeds 85% and ancillary revenue scales) is added to the Year 1–3 cumulative position.
 - **Value-creation framing:** by Year 3 the Center carries an annualised EBITDA run-rate of $\approx$ EGP 10.9M; at a 3–4 $\times$ healthcare-services exit multiple this implies an indicative enterprise value of **EGP 33–43M by Year 3**, rising toward or beyond the EGP 52M invested by Year 4–5 — the appropriate lens for a flagship brand-building investment, where strategic and brand value (and optionality for franchising/expansion) supplement pure cash payback.
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22. Consolidated Revenue Streams (all options)

1. Clinical session fees (per-session, package, or course pricing) — the core revenue driver across all three departments
2. Initial-assessment fees (typically priced at a premium to a standard follow-up session)
3. Insurance/corporate reimbursement and direct-billing contracts
4. Corporate wellness & occupational-health contracts (Option 2/3)
5. Membership/subscription wellness packages (Option 3)
6. Medical-tourism packages (Option 3)
7. Training academy / continuing-education courses for external therapists (Option 2/3, Year 2+)
8. Home-programme retail (orthotics, sensory tools, home-exercise equipment) (Option 2/3)
9. Telehealth follow-up consultations (all options, Year 2+)

23. Risk Assessment

Risk	Likelihood	Impact	Mitigation
Regulatory/ licensing delay	Medium	High (delays opening, extends pre-revenue cash burn)	Engage licensing consultant early; begin MOHP application in parallel with fit- out; build a 2- month buffer into the timeline
Key-person dependency on three founders	Medium	High	Document clinical protocols; cross- train senior staff as deputies; key- person insurance
EGP depreciation raising import/equipment costs	High	Medium–High	Source equipment quotes in USD with a contingency line (10–15%) already built into estimates; phase luxury/imported equipment purchases
Staff retention (PT/OT emigration to Gulf)	High	Medium	Competitive compensation with growth path to department leadership; CPD investment; equity/profit- share for senior long-tenure staff (Option 2/3)

Risk	Likelihood	Impact	Mitigation
Slower-than-modelled patient ramp-up	Medium	High (cash-flow strain in Year 1–2)	Working-capital reserve sized at 3–6 months opex; flexible staffing (part-time/locum) in Year 1
Insurance/corporate reimbursement delays	Medium	Medium	Require co-pay/upfront patient portion; factor 30–45 day AR cycle into cash-flow planning
Competitive entry (hospital groups expanding rehab lines)	Medium	Medium	Differentiate via multidisciplinary “one roof” positioning and founder personal brand, which is harder to replicate quickly
Equipment breakdown (esp. robotics/VR, Option 3)	Low–Medium	Medium	Vendor service contracts, preventive-maintenance schedule, in-house biomedical technician (Option 3)
Economic downturn reducing discretionary luxury spend	Low–Medium	Medium (Option 3 specific)	Diversify Option 3 revenue via insurance-eligible core sessions alongside luxury add-ons; corporate contracts as a

Risk	Likelihood	Impact	Mitigation
			stabiliser

24. Expansion Strategy

- **Phase 1 (Year 1–2):** establish the flagship Fifth Settlement center, build referral networks per department, achieve break-even.
- **Phase 2 (Year 2–3):** add department-level service-line extensions (speech therapy, sports-performance testing, robotic/VR neuro-rehab) as demand and cash flow allow; pursue insurance-panel and corporate contracts.
- **Phase 3 (Year 3–4):** open a second branch in another high-growth Greater Cairo corridor (e.g., Sheikh Zayed/6th of October on the Giza side, or the New Administrative Capital) replicating the three-department model or a focused single-department satellite.
- **Phase 4 (Year 4–5):** consider a therapist-training academy/CE-course revenue line leveraging founder reputations; explore tele-rehabilitation for follow-up care beyond Greater Cairo; evaluate a fourth department (e.g., cardiopulmonary rehabilitation or women’s health/pelvic-floor therapy) based on demand signals.
- **Phase 5 (Year 5+, primarily Option 3 path):** formalise a franchise/licensing model for the TRIAD brand; deepen medical-tourism partnerships; evaluate strategic acquisition interest from a hospital group or regional healthcare platform as an exit/liquidity option for founders and investors.

25. Implementation Timeline

Phase	Months	Key activities
1. Company formation & planning	1–2	GAFI incorporation, partnership/shareholder agreement, feasibility study finalisation, site shortlisting
2. Site selection & lease	2–3	Lease negotiation and signature, architectural survey, MOHP pre-

Phase	Months	Key activities
3. Licensing application	3–6 (parallel)	application consultation MOHP facility license, syndicate registrations, fire-safety/civil-defence approval, tax and commercial registration
4. Design & fit-out	3–7	Architectural/interior design finalisation, construction/renovation, MEP works, (Option 3) pool construction
5. Equipment procurement	4–7	Vendor selection, ordering (allow lead time for imported items), installation, calibration
6. Staff recruitment	5–7	Department Head confirmation, senior- therapist and support- staff hiring, background/syndicate verification
7. Technology setup	6–7	EMR/software installation, website, telehealth setup, staff training on systems
8. Pre-launch marketing	6–8	Brand finalisation, website launch, digital campaign build, referral- partner outreach begins
9. Soft opening	8	Limited-capacity soft launch, staff workflow testing, friends-and- family/community screening events
10. Grand opening	9	Public launch event, full marketing campaign

Phase	Months	Key activities
11. Ramp-up & optimisation	9–24	activation Referral-network build-out, service-line additions, first performance review at month 12

(Option 1 can compress this timeline to ≈6–7 months to opening given lighter fit-out and equipment lead times; Option 3 may extend to 10–12 months given pool construction and imported robotics/VR equipment lead times.)

26. Appendix — Key Assumptions & Disclaimers

- All financial projections are illustrative planning estimates prepared for early-stage investor discussion. They are not a guarantee of future performance and should be validated through a formal feasibility study, independent architectural/MEP quotation, and vendor procurement quotes before capital commitment.
- Exchange rate used: USD 1 = EGP 50 (indicative, July 2026); Egypt's currency has shown material short-term volatility and investors should stress-test all USD-equivalent figures.
- Session pricing assumptions are benchmarked against publicly observable Cairo private-clinic pricing (approximately EGP 300–600 for standard physiotherapy sessions and EGP 100–1,300+ for specialist/occupational therapy sessions in 2025–2026), adjusted upward for the Fifth Settlement's premium demographic and for TRIAD's senior-clinician-led positioning.
- Rent assumptions are benchmarked against observed Fifth Settlement clinic listings (≈EGP 500–580/m²/month for finished clinic space in 2026), with Option 1 assuming a secondary-street location and Options 2–3 assuming premium corridor locations.
- Payroll figures are indicative planning-level averages for Cairo private-healthcare compensation in 2026 and should be validated against current market benchmarking before finalising an offer letter.
- This document does not constitute an offer to sell securities. Any investment would be subject to definitive legal agreements, due diligence, and applicable Egyptian company and securities law.